

A Case of Angioedema Associated with Amlopril (Cardiozin) Therapy: A Case Report and Literature Review

R. Desai MD (1), S. Nair MD (2) | (1) Dept. of Internal Medicine, Test Medical College (2) Dept. of Dermatology, Test Medical College

Abstract: Angioedema is a rare but serious adverse effect associated with several antihypertensive agents. We describe a case of a 62-year-old woman who developed facial and lip angioedema seven days after initiating amlopril (Cardiozin) 10 mg once daily. Symptoms resolved with discontinuation of the drug and supportive treatment. This report also reviews existing literature on class-related angioedema risk. (Note: this is a fictional case created for software testing purposes.)

Introduction

Drug-induced angioedema is a recognized but uncommon adverse reaction, most classically associated with ACE inhibitors, but also reported with other antihypertensive classes including some calcium channel blockers and combination agents. Clinicians should maintain a high index of suspicion when new swelling develops after initiation of antihypertensive therapy, particularly within the first two weeks of treatment, as delayed recognition can result in airway compromise.

Case Presentation

A 62-year-old woman with an eight-year history of hypertension and a five-year history of type 2 diabetes mellitus was started on amlopril (Cardiozin) 10 mg orally once daily for blood pressure control. She had no prior history of angioedema or known drug allergies. Seven days after starting therapy, she presented to the emergency department with acute swelling of the face and lips accompanied by mild dyspnea. On examination, she had non-pitting edema of the lips and periorbital region without urticaria. Laboratory workup showed mildly elevated serum tryptase and eosinophilia, with normal C4 complement, favoring a non-hereditary, drug-related mechanism. She was treated with intravenous antihistamines and corticosteroids and observed for 48 hours, with complete resolution of symptoms. The suspect drug was discontinued and she was transitioned to an alternative antihypertensive class.

Discussion

While angioedema is a well-documented class effect of ACE inhibitors, occurring in an estimated 0.1-0.7% of patients, reports of angioedema with other antihypertensive classes are less common and the underlying mechanism is not fully elucidated. Proposed mechanisms include bradykinin accumulation and mast-cell mediated pathways. Clinicians prescribing any new antihypertensive agent should counsel patients about early signs of angioedema and advise immediate discontinuation and medical review if swelling occurs.

Conclusion

This case adds to the limited literature describing angioedema associated with amlopril and reinforces the importance of pharmacovigilance reporting for early-onset swelling reactions following initiation of antihypertensive therapy, even outside the ACE inhibitor class.

References

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